

DECAFED LINCTUS

DESCRIPTION: A clear red colour syrup with raspberry flavour

COMPOSITION: Each 5 ml contains:

Pseudoephedrine Hydrochloride	30.0 mg
Triprolidine Hydrochloride	1.25 mg
Dextromethorphan Hydrobromide	15.0 mg

ACTIONS:

Pseudoephedrine hydrochloride is an orally active and indirect sympathomimetic agent; that is, it releases norepinephrine from adrenergic nerves. It acts on alpha-adrenergic receptor in the mucosa of the respiratory tract, producing vasoconstriction. It shrinks swollen nasal mucous membranes; reduces tissue hyperemia, edema, and nasal congestion; and increases nasal airway patency. Drainage of sinus secretions may be increased and obstructed eustachian ostia may be opened. It has the potential for excitatory side effects. At the recommended oral dose, it has little or no pressor effect in normotensive adults. Triprolidine is an antihistamine (H₁-receptor). It acts by competing with histamine for H₁-receptor sites on effector cells. It prevents, but does not reverse, responses mediated by histamine alone. It also has anticholinergic actions and provides a drying effect on the nasal mucosa.

Dextromethorphan is an antitussive. It suppresses the cough reflex by a direct action on the cough center in the medulla of the brain.

PHARMACOLOGY:

Pseudoephedrine is readily absorbed from the gastrointestinal tract. Elimination is renal and about 55 to 75% of a dose is excreted unchanged. Thus, it may accumulate in patients with renal insufficiency. It is incompletely metabolized in the liver, onset of action within 15 to 30 minutes, time to peak effect within 30 to 60 minutes and has a duration of action for 3 to 4 hours. The rate of excretion is accelerated in acidic urine. It has a mean elimination half-life of 4-6 hours which is highly dependent upon urine pH. The elimination half-life is decreased at urine pH lower than 6 and may be increased at urine pH more than 8. Steady state maximum plasma concentration of pseudoephedrine is 498 ± 129 ng/mL. The terminal half life, oral clearance and apparent volume of distribution are 6.2 ± 1.8 hours, 5.9 ± 1.7 mL/min/kg, and 3.0 ± 0.4 L/kg, respectively. It does not bind to human plasma protein over the concentration range of 50 to 2000 ng/mL. Small amounts are distributed into breast milk. The rate or extent of pseudoephedrine absorption is not affected by food. Triprolidine is absorbed from the gastrointestinal tract. The time to peak effect is 2 to 3 hours, half life of 3 to 3.3 hours and a duration of action of 4 to 8 hours. A carboxylated derivative accounts for about half the dose excreted in the urine. Dextromethorphan is rapidly absorbed from gastrointestinal tract and exerts its effect in 15 to 30 minutes. The duration of action is approximately 3 to 6 hours. It is metabolized mainly by liver enzymes undergoing O-demethylation, N-demethylation, and partial conjugation with glucuronic acid and sulfate. In

human, (+)3-hydroxy-N-methylmorphinan, (+)-3-hydroxymorphinan, and traces of unmetabolized drug were found in urine after oral administration.

INDICATIONS:

Symptomatic relief of upper respiratory tract disorders which are benefited by a combination of an antihistamine, a nasal decongestant and an antitussive.

CONTRAINDICATIONS:

Contraindicated in patients with known hypersensitivity to pseudoephedrine, triprolidine or dextromethorphan; in patients under treatment with monoamine oxidase (MOA) inhibitor therapy or within two weeks of stopping such treatment. It is also contraindicated in patients with narrow-angle glaucoma or urinary retention, in those with severe hypertension, or severe coronary artery disease, and in patients who have shown hypersensitivity or idiosyncrasy to its components, to adrenergic agents, or to other drugs of similar chemical structures. Manifestation of patient idiosyncrasy to adrenergic agents include: insomnia, dizziness, weakness, tremor, or arrhythmias. Also contraindicated in patients with peptic ulcer and during an asthmatic attack.

SIDE EFFECTS / ADVERSE REACTIONS:

Pseudoephedrine hydrochloride may cause mild CNS stimulation in hypersensitive patients. Nervousness, excitability, restlessness, dizziness, weakness, or insomnia may occur. Headache, drowsiness, tachycardia, palpitation, pressor activity, and cardiac arrhythmias have been reported. Sympathomimetic drugs have also been associated with other untoward effects such as fear, anxiety, tenseness, tremor, hallucinations, seizures, pallor, respiratory difficulty, dysuria, and cardiovascular collapse.

Adverse effects with dextromethorphan appear to be rare and may include slight drowsiness, dizziness, gastrointestinal disturbances, nausea or vomiting and stomach pain. Toxic psychosis (hyperactivity, visual and auditory hallucinations) has been reported after ingestion of 300mg or more of dextromethorphan. Respiratory depression has been reported to occur with very high doses. Dextromethorphan abuse and dependence may occur rarely, especially following prolonged use of high doses.

The most common side-effect of the sedating antihistamines including triprolidine hydrochloride is CNS depression, with effects varying from slight drowsiness to deep sleep, and including lassitude, dizziness, and incoordination (although paradoxical stimulation may occasionally occur, especially in children). These sedative effects, when they occur, may diminish after a few days after treatment. Other more common side-effects include headache, psychomotor impairment, and antimuscarinic effects, such as dry mouth, thickened respiratory-tract secretions, blurred vision, urinary difficulty or retention, constipation, and increased gastric reflux. Occasional gastrointestinal side effects are nausea, vomiting, diarrhoea, or epigastric pain. Palpitations and arrhythmias have been reported occasionally. It may sometimes cause rashes

and hypersensitivity reactions (including bronchospasm, angioedema, and anaphylaxis) and cross-sensitivity to related drugs may occur. Blood disorders, including agranulocytosis, leucopenia, haemolytic anaemia, and thrombocytopenia, although rare, have been reported. Other adverse effects include convulsions, sweating, myalgia, paraesthesias, extrapyramidal effects, tremor, sleep disturbances, depression, tinnitus, hypotension and hair loss.

PRECAUTIONS/WARNINGS:

Because of its pseudoephedrine content, it should be used with caution in patients with hypertension, diabetes mellitus, ischemic heart disease, increased intraocular pressure, hyperthyroidism, renal impairment, or prostatic hypertrophy. Sympathomimetic amines may produce CNS stimulation with convulsions or cardiovascular collapse with accompanying hypotension.

Antihistamines may cause excitability, especially in pediatric patients. Nervousness, dizziness or sleeplessness may occur at dosages higher than the recommended dose. Dextromethorphan should be used with caution in sedated patients, in the debilitated, and in patients confined to the supine position.

Not suitable to be used in children below 2 years old.

For children between 2 to 6 years old — use with caution and follow the advice given by the doctor or pharmacist.

USE IN PREGNANCY AND LACTATION:

Although pseudoephedrine, triprolidine and dextromethorphan have been in widespread use for many years without apparent ill consequences, there are no specific data on their use during pregnancy. Caution should therefore be exercised by balancing the potential benefit of treatment against any possible hazards.

It has been estimated that approximately 0.5 to 0.75% of a single dose of pseudoephedrine and approximately 0.06 to 0.2% of a single dose of triprolidine ingested by a mother will be excreted in the breast milk over 24 hours, but the effect of this on breast-fed infants is not known. It is not known whether dextromethorphan or its metabolites are excreted in human milk.

ABILITY TO DRIVE AND USE MACHINES:

May cause drowsiness and impair performance in test of auditory vigilance. Patient should be advised not to drive or operate machinery until they have determined their own response.

DRUG INTERACTIONS:

It is contraindicated in patients taking monoamine oxidase inhibitors and for 14 days after stopping use of an MAO inhibitor. Concomitant use with antihypertensive drugs which interfere with sympathetic activity (eg. Methyldopa, mecamylamine, and reserpine) may reduce their antihypertensive effects. Increased ectopic pacemaker activity can occur when pseudoephedrine is used concomitantly with digitalis. Combined effects on the cardiovascular system may be harmful to the patient. Thus, care should be taken in the administration of Defaced Linctus concomitantly with other sympathomimetic amines. Do not take this product if you are presently

taking a prescription drug for high blood pressure or depression, without first consulting a physician.

DOSAGE AND ADMINISTRATION:

Adults and children > 12 years

10 ml three times a day

Children 6 — 12 years

5 ml three times a day

Children 2 — 6 years

2.5 ml three times a day

ROUTE OF ADMINISTRATION:

Oral administration

SYMPTOMS AND TREATMENT FOR OVERDOSAGE:

Symptoms:

May include drowsiness, lethargy, dizziness, ataxia, nystagmus, weakness, hypotonicity, respiratory depression, dryness of the skin and mucous membranes, tachycardia, hypertension, hyperactivity, hyperpyrexia, irritability, visual and auditory hallucinations, convulsions, difficulty with micturition, nausea and vomiting.

Treatment:

Measures should be taken to maintain and support respiration and control convulsions. Gastric lavages should be performed. Catheterisation of the bladder may be necessary. If desired, the elimination of pseudoephedrine can be accelerated by acid diuresis or by dialysis. Naloxone has been used successfully as a specific antagonist to dextromethorphan.

PRESENTATION:

Bottles of 60ml and 120 ml

STORAGE:

Store in dry place below 30°C. Protect from light.

Keep the container tightly closed.

Keep out of reach of children .

Jauhkan dari kanak-kanak .

MANUFACTURED BY:

Noripharma Sdn Bhd
Lot 5030, Jln Teratai,
5 1/2 Mile off Jalan Meru,
41050 Klang,
Selangor Darul Ehsan.

DATE OF REVISION:

July 2022